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Name: حسنات زينهم عبده عبدالرحمن National ID: 27904012103369	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حسبات
زينهم عبده عبد الرحمن
بنى سلامة
منشأة القناطر - الجيزة

٢٧٩٠٤٠١٢١٠٣٣٦٩

JP3593093

٢٠٢٢/١٠ ٢٧٩٠٤٠١٢١٠٣٣٦٩

عاملة زراعية
مسلمة
أنثى

البطاقة سارية حتى ٢٠٢٩/١٠/٠٨

DM

منزله من الامارات والفضوليات



جهة الإصدار: المربع الجديد

تاريخ الإصدار: 2013-1-25

إلى من يهمه الأمر:

المادة العامة للتأمين الصحي ككل من خلال قانون منظمة للاقتصاد والمعلومات التأمين
الإحصائي رقم 99 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القانون العام والمعلومات
والإحصائي" و كذا قانون 99 لسنة 1992 الخاص باللائحة التنفيذية و قانون رقم 86 لسنة 2012
الإحصائي دون النص التوضيحي. ولقانون رقم 23 لسنة 2012 الخاص باللائحة التنفيذية
وطريقة للاعداد والمعلومات العامة من التأمين والإحصائي
وطريقة للاعداد والمعلومات العامة من التأمين والإحصائي
والجديد كدين إين

المرجع (أ) حركات (أ) رقم 27904012103369
وطريقة التي يطلب من التأمين بولك معطى التأمين
بموجب من التأمين كذا الملائحة التنفيذية التأمين
والجديد

تاريخ الإصدار: 2013-1-25



المركز المركزي للمعلومات والقرارات
رقم القرار



المادة العامة للتأمين الصحي ككل من خلال قانون منظمة للاقتصاد والمعلومات التأمين
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والجديد كدين إين

الهيئة العامة للتأمين الصحي

Name: حسنات زينهم عبده MR. : 901	
Detailed Medical Report	
السيد	
تحية طبية وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه 45 Y PREM MBC TO BRAIN IDC ER6/8 PR7/8 HER23+ HISTORY RT BREAST CANCER REC ADJ HORMONAL TT AND HERCEPTIN TILL 5/2023 BRAIN METS LOCALIZED RT TO BRAIN FRONTAL SOL XELODA ,TYKERB TILL2/2024 ZOLADEX FEMARA TYKERB TILL 6/2024SUSP LESION IN LEFT FEMORAL NECK TURNED TO BE AVN SHIFTED TO HERCEPTIN AROMASIN ZOLADEX 6/2024 PD OF RT FRONTAL SOL EF REC SRS 7/2024, 7/2025 DP OF FRONTAL LESION rec srs again FOR NEW CTS MRI BRAIN FOR C29 HERCEPTIN 408 AND AROMASIN ZOLADEX DUE 17/3 cts bs mri brain with comparison pending. ef 64% on 30/12/2025 for c30 herceptin ,zometa aromasin MRI BRAIN :REGRESSIVE COURSE +SOL WITH HEMORRAGIC COMPONENT ???(METASTIC). FOR AROMASIN +AND C31 HERCEPTIN IV+ZOLADEX Pt is a known case of metastatic breast cancer with brain metastasis, currently on Aromasin + Herceptin. Latest imaging: · CT chest: No evidence of metastatic disease. · CT pelvis/bone: Left femoral head fracture with underlying sclerosis → ? metastatic vs degenerative/post-traumatic changes. Assessment: MRI pelvis , bone scan >> Pending FOR AROMASIN +AND C32 HERCEPTIN IV+ZOLADEX bone scan 4/2026 intense left acetabulum left femoral neck (history of left femoral neck lesion by mri osteonecrosis) small focal areas medial end of rt clavicle and sternum representing early metastasizing disease .cts retrocaval ln 12x8 left pulmonary nodule 5.6mm .left femoral fracture for mri left hip joint and left si joint for mdt orthopedic consult meanwhile continue herceptin for c32 herceptin 400(wt66)and aromasin mri brain2/2026sd of rt frontal sol m 3.4x2.7x3.2 sm free mdt on 6/5/2026 for navelbine tykerb and trial of enhertu from ngo .pending approval ef 66% 24/3/2026 for herceptin 400 zometa aromasin FOR C1 NAVELBINE D1 D8 WT 150 HT 65 SA 1.6 SPT CLAIMS SHE STILL HAS APPROVAL OF HERCEPTIN EVEN THOUGH TYKERB IS ALSO APPROVED? FOR CONTINUATION OF HERCEPTIN TILL END OF APPROVAL PATIENT FOR HERCEPTIN IV , ZOLADEX pending tykerb approval then to continue on navelbine tykerb Today for tykerb 5*1*30 + Navilbine 100 mg D1 D8 + supp ttt labs >>> OK PT CLAIMS SHE STILL HAS APPROVAL OF HERCEPTIN EVEN THOUGH TYKERB IS ALSO APPROVED? FOR CONTINUATION OF HERCEPTIN TILL END OF APPROVAL TODAY FOR HERCEPTIN IV ONLY (RECEIVED NAVELBINE LAST WEEK)	
تحريراً في ,	
التاريخ :	19-09-2026
الطبيب المعالج :	dina makram
يعتمد ,	

(MR 901 (continued, page 2/2 - حسنات زينهم عبده

ef 64%13/7 for herceptin 400 ca vit d ondalez (no frank progression bec bs equivocal lesions in sternum rt clavicle left femoral lesion (previous known AVN) for new cts

cts 3/8/2026 newly developped pretracheal ln m35x17 mm pulmonary nodules in left upper lobe m 6.4mm referral for tdm1 or enhertu pending for continuation of herceptin this cycle only 9TILL APPROVAL OF KADCYLA OR ENHERTU FROM NGO) IF STILL NO APPROVAL NEXT TIME TO ST TYKERB AS NEW CTS DP FOR HERCEPTIN 400 ZOMETA

Today for tykerb 5*1*30 + Navilbine 90 mg D1 D8 + supp ttt labs >>> OK



مملكة العربية السعودية
وزارة الصحة العامة
لحفظ صحة المرأة السعودية



01/06/2020

MDT FORM

Name:	Imed Zahrani Alsh	Gender:	903
Age:	47 Y	ID:	27904012103369
Marital Status:	Married	Phone:	01203918440
NRS SCORE:	10.5	Marital Status:	
CO Morbidity:		Family History:	
		Reason of MDT:	

Clinical Data:

DX: ☐ Hc ☐ R ☐ L

ER: ☐ 6/9 ☐ PR: ☐ 7/9 ☐ HER2: ☐ 3+ ☐ Ki67: ☐

Sonohistology:

Histology:

4.5 Y HPE NEC TO BRAIN WITH SUSPECTED NEW BONE METS RECENTLY. IDC ER+/H PR+/H HER2+ HISTORY RT SURGICAL CANCER. CH IL. NO REC NED 4G. A Tissue Perceptin rt arm or rec tamoxifen herceptin adjuvant ended 2/2021. TIL 5/2021 BRAIN METS. RT frontal single ad). LOCALIZED RT TO BRAIN FRONTAL SOL. XELODA. TYNIBO TIL2/2024 ZOLADEX TYNIBO TIL 6/2024 SOL. LESION IN LEFT TEMPORAL. METS TURNED TO RT AND SHIFTED TO HERCEPTIN AROMASIN ZOLADEX 6/2024 PO OF RT FRONTAL SOL. METS 7/2024 continued herceptin aromatase inhibitors 7/2025 OP OF FRONTAL LESION rec. second scan. RT rec up to 32 cycles herceptin bone scan 4/2026 internal left acetabulum left temporal neck. History of left temporal neck lesion by mri MVI. Small focal areas metastatic end of RT clavicle and sternum. Reproducting early metastatic disease. CTS metastatic in 12th rib pulmonary nodule 5.6mm. Left temporal fracture for mvi left hip joint and left 5 joint. mvi brain/202066 of RT frontal sol in 3.4x2.7x3.2 cm free FOR MDT

Recommendations:

MDT Outcome & Plan

For Ertenu (see MDT) intervention for Tynib- Nivoluma

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	NUTRITION	PSYCHIATRY



Name : Hasanat Zeinhom Abdu Abdulrahman

Date : 20-04-2026 07:49

Branch : Dokki

Medical No. : 354598

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاليد مستشفىات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female, 47 years old. History of right breast carcinoma (diagnosed 2020); generalized bone aches. Evaluation for potential skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time. Additional spot views were obtained when necessary.

Findings:

- o Intense, focal tracer localization is identified involving the **left acetabulum and the left femoral neck**. The intensity and location are highly suspicious for a metastatic deposit with a **high risk of impending pathological fracture**.
- o Small focal areas of increased tracer uptake are noted at the **medial end of the right clavicle** and the **sternum**. These findings are suggestive of early developing metastatic deposits.
- o No other focal "hot" or "cold" lesions are identified in the skull, ribs, or remaining long bones.
- o Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- o **Picture highly suggestive of Multifocal Metastatic Disease** at the left acetabulum/femoral neck, sternum, and right clavicle as described, for radiological correlation and follow up.
- o **Pathological Fracture Risk** at the left hip (a high risk of impending fracture), for urgent orthopedic and oncological correlation for the left hip.

Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name: Hasanat zenhom abdou
Date: 2026-03-24
Referred by: Prof.Dr. cap+e
Hospital: Goustave Roussey International Egypt

CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

In comparison to previous report dated 14/6/2025 current study reveals:

- Right mastectomy with clear operative bed.
- No sizable suspicious lymphadenopathy.
- Retrocaval lymph node measures 12x8 mm.
- Left lingual pulmonary nodule measures 5.6 mm.
- Right upper lobar subpleural reticulations could be post therapeutic changes.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Right mastectomy with clear operative bed.
- No sizable suspicious lymphadenopathy.
- Retrocaval lymph node .
- Left lingual pulmonary nodule likely metastatic.
- Right upper lobar subpleural reticulations could be post therapeutic changes.

Patient's Name: Hasanat zenhom abdou
Date: 2026-03-24
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- Left femoral head fracture with sclerosis.
- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

Left femoral sclerosis and fracture for bone scan.

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq

Name : Hassant zenhom abdou abd elrahman

Date : 03-02-2026 11:52

Branch : Portable

Medical No. : GRI-

260203114843STD131221107523748902

Ref.Physician Prof : BRAIN +C

Ins.Company :

MRI OF THE BRAIN

Technique :

Axial T1, T2 ,FLAIR, DWI& ADC

Coronal T2

Sagittal T2

Postcontrast T1

Findings:

As compared to last study dated 29-7-2026 , current study reveals

- Mild regressive rather Stationary course as regard the previously noted right frontal area of altered MR signal measuring 3.4 x2.7 x3.2 cm in the CC , TS & AP diameter compared to 3.6 x3.2 x2.6 cm In the last study , the mass lesion eliciting low T1, bright T2 and FLAIR WI signal with central area of bright T1 and low T2 WI signal, denoting hemorrhagic component. It showing heterogenous, mainly peripheral contrast uptake. it is seen associated with surrounding edema and causing mild mass effect in the form of effacement of the adjacent cortical sulci.
- Normal size and configuration of the supra and infra tentorial ventricular system
- No midline shift or deformity
- No cerebellar or brain stem areas of altered MR signal.
- No extra axial collections.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

OPINION:

- *Rather mild regressive than stationary curse as regard the Right frontal area of altered MR signal and SOL with hemorrhagic component as described ? metastatic, for MR spectroscopy.*

Dr Abubakr Hejazi MSc FEBR

Dr. Sherif Wagdy

DOC GRI

⑥

نمایند

خريطة رقم (4) - الدور الأول - مبنى رقم (957) - إمدادات الماء - معبر القريعة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 957 - Nile Corniche - Near El Qalaa - Cairo
Egypt



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m7189

Female :	المرأة :	40 Y :	40 سنة :
03/12/2010 10:26 :	التاريخ المستلم :	25/12/2019 09:27 :	تاريخ الاكبر :
			مركز اورام دار السلام :
			مركز اورام دار السلام :

Primary Sample :

Paraffin Block

Test :

ER, PR & HER2 neu (ER&PR : Harvey scoring system - Her2neu : CAP scoring system)

Comment :

Technique :

sample received was treated by CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody - CONFIRM anti-
progesterone receptor (PR) (H2) Rabbit monoclonal primary antibody.
sample received was treated by VENTANA anti-HER2/neu (4B6) Rabbit monoclonal primary antibody using DAB Detection
in Ventana benchmark CX

Result :

ER: moderate nuclear staining of 80% of tumour cells
PR: moderate nuclear staining of 80% of tumour cells
HER2/neu protein over expression: strong continuous membranous staining of > 10% of tumor cells

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 6/8)
PR: POSITIVE (SCORE 7/8)
HER2/NEU PROTEIN OVER EXPRESSION: POSITIVE (SCORE 3+)

Pathologist:

Lab Director:



Patient Name: Hosanah Zenhoni Abdou Abd Al Rahman

Date: 29/07/2025

Exam: MRI BRAIN WITH CONTRAST

Age: 40 Year(s)

Medical ID: 4165/2019

Pat. ID: 12686

MRI EXAMINATION OF THE BRAIN

MRI TECHNIQUE:

Axial T1, T2 & FLAIR WIs

Sagittal T1 WIs

Post contrast axial sagittal and coronal T1 WIs

** A reported case of breast cancer.

** Compared to the previous study report dated 4/1/2025.

MRI FINDINGS:

Compared only to the previous report measurements, progressive course regarding the size of the right frontal intra-axial space occupying lesion seen measures about 32 x 36 x 26 mm compared to 22x16x16 mm on the previous report, still eliciting hypo/iso-intense T1, relatively increased T2 and FLAIR signal intensity, showing heterogenous enhancement in the post contrast study.

Still noted enhancement of the adjacent dura.

Normal appearance of the cerebellum, brain stem and cranio-cervical junction.

Normal size and configuration of the ventricular system.

No shift of the midline structures.

OPINION:

Compared only to the previous report measurements, progressive course, for clinical correlation.

Dr. Zeyad Badawy, MSc, FRCR,
Radiology consultant.

[Signature]

Patient Name: Hasanat Zenhom Abdou Abd Al Rahman
Exam: CT CAP+C (CT Chest Abd. Pelvis)
Medical ID:

Date: 30/08/2025

Age: 46 Year(s)

Pat. ID: 12686

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Clinical History:

- > Known case of right breast cancer.

CT SCAN OF THE CHEST



Findings:

- > Evidence of right MRM with no sizable soft tissue mass lesions to account for residual/recurrent neoplasia.
- > No sizable right axillary lymph nodes.
- > Few left axillary lymph nodes, the largest seen measuring about 14.5x10 mm.
- > Few prominent mediastinal lymph nodes are noted, the largest seen pretracheal/retrocaval measuring about 10x7 mm.
- > Few left lung pulmonary nodules, the largest is noted at the lingula measuring about 7 mm in diameter.
- > Few right upper and middle lobar subpleural reticulations are noted, could be post radiotherapy sequel.
- > Patent tracheo-bronchial tree.
- > Normal CT appearance of the mediastinal vessels.
- > Average size of the heart.
- > No evidence of pleural or pericardial effusion.

OPINION:

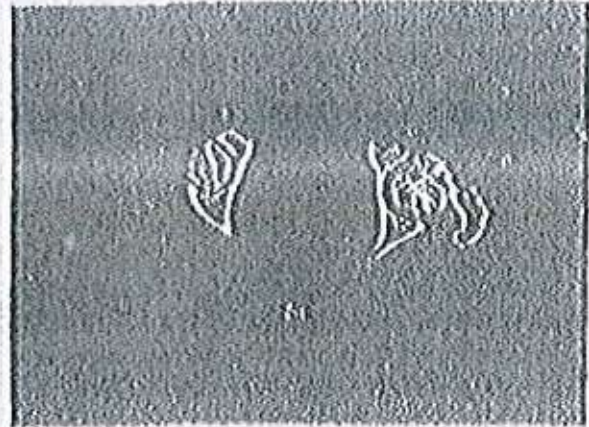
- > Evidence of right MRM with no sizable soft tissue mass lesions to account for residual/recurrent neoplasia.
- > No sizable right axillary lymph nodes.
- > Few left axillary lymph nodes.
- > Few prominent mediastinal lymph nodes.
- > Few left lung pulmonary nodules.
- > Few right upper and middle lobar subpleural reticulations, could be post radiotherapy sequel.

CT SCAN OF THE ABDOMEN AND PELVIS:

- > Compared to the previous study report dated 14-6-2025, the current study revealed:

Findings:

- The liver is mildly enlarged (16.5 cm) with diffuse low homogenous CT attenuation. No sizable focal lesions.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size of the spleen (9.5 cm) with no focal lesions seen.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Still noted few tiny left paraaortic lymph nodes.
- Average filling of the urinary bladder with no sizable stones or masses.
- Left femoral head lytic lesions with partial compression for correlation with the unavailable previous films.



Impression:

- Fatty hepatomegaly for clinical and lab correlation.
- Left femoral head lytic lesions with partial compression for correlation with the unavailable previous films.

Marina Bssam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
04/09/2025 1:39 AM

Z, H Patient ID: 123021214 DOB: 01-Apr-1979 Date: 16-Feb-2025
Patient Lastname: Z
Patient Firstname: H
HIS Patient ID: 123021214
RIS Patient ID: 339067
Date of Birth: 01/04/1979

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

REPORT TEXT

99mTc MDP Bone Scan Report (16/02/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

Compared to previous bone scan dated 27/5/2024 the current study revealed:

- " Still seen diffuse tracer uptake is seen at left acetabulum, head, neck and trochanteric area of left femur ?? chronic AVN (in the view of the patient history of corticosteroid intake)?? metastatic, both are postulated
- " Otherwise, no definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

- " Still noted solitary active uptake at left hip??AVN?? Metastatic for local MRI correlation.
- " Otherwise, no definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.

Much Obligated
Omnia Talaat, MD

REPORT TEXT ENDE